**Clinical Study Summary Report (CSSR) Document Status:** Final | **Version:** 1.0 | **Date:** 06-Apr-2026

---

## 1. Administrative & Study Identification

**Brief Study Title:** A Study to Compare the Efficacy and Safety of BMS-986489 (BMS-986012+ Nivolumab Fixed Dose Combination) in Combination With Carboplatin Plus Etoposide to That of Atezolizumab With Carboplatin Plus Etoposide as First-Line Therapy in Participants With Extensive-Stage Small Cell Lung Cancer (TIGOS). **Full Study Title:** A Randomized, Double Blind, Multicenter Phase 3 Trial of BMS-986489 (BMS-986012+Nivolumab Fixed Dose Combination) in Combination With Carboplatin Plus Etoposide vs Atezolizumab in Combination With Carboplatin Plus Etoposide as First-Line Therapy in Participants With Extensive-Stage Small Cell Lung Cancer (TIGOS) **Short Title/Acronym:** TIGOS Study **Protocol Number:** CA245-0001 **NCT Number:** NCT06646276 **Trial Status:** RECRUITING **Sponsor Name:** Bristol Myers Squibb **Company:** Bristol Myers Squibb **Investigational Product:** BMS-986489 (Atigotatug/anti-fucosyl-GM1 [BMS-986012] + Nivolumab Fixed Dose Combination) **Phase of Development:** PHASE3 **Medical Monitor:** Clinical Trial Medical Director, Bristol Myers Squibb

---

## 2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To compare the efficacy (Overall Survival) of BMS-986489 in combination with carboplatin plus etoposide versus atezolizumab with carboplatin plus etoposide as first-line therapy in participants with Extensive-Stage Small Cell Lung Cancer (ES-SCLC). **Secondary Objectives:** To evaluate time to definitive deterioration (TTDD), objective response rate (ORR), duration of response (DoR), progression-free survival (PFS), and overall safety profile.

**2.2 Background & Rationale** Extensive-Stage Small Cell Lung Cancer (ES-SCLC) is an aggressive malignancy with limited treatment options and a median overall survival often remaining under 1 year. Fucosyl-monosialoganglioside-1 (fuc-GM1) is highly expressed in 50%–90% of SCLC tumors but minimally in normal tissues. In prior phase 2 studies, combining the innate immune inducer atigotatug (anti-fuc-GM1) with nivolumab and chemotherapy showed promising survival benefits over standard immunotherapy. This study is conducted to confirm those findings against the current standard of care (atezolizumab + chemotherapy) to establish a new, highly targeted first-line therapy for ES-SCLC patients.

---

## 3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Active Comparator (Atezolizumab + Carboplatin + Etoposide) **Blinding:** Double-blind **Randomization:** 1:1 (Stratified by ECOG performance status, presence of liver metastases, and presence of brain metastases)

**3.2 Planned Enrollment & Duration Targeted Enrollment:** 530 participants **Number of Sites:** Approximately 182 sites across 26 countries **Estimated Study Start:** 25-Feb-2025 **Estimated Primary Completion:** 06-Apr-2028

---

## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Age $\ge 18$ years. 2. Participants must have diagnosis of Extensive-Stage Small Cell Lung Cancer (ES-SCLC). 3. Participants must be Healthy enough to do their normal activities with little or no help based on the ECOG performance scale. 4. Participants must have at least one tumor that can be measured using special imaging techniques like a CT scan or MRI at a site other than the brain and nervous system. 5. Any previous limited-stage SCLC treatment completed $\ge 6$ months prior to study treatment initiation. 6. Other protocol-defined Inclusion criteria apply.

**4.2 Exclusion Criteria** 1. Participants have already received certain types of treatment for extensive stage small cell lung cancer (Prior systemic treatment for ES-SCLC). 2. Participants have certain health conditions, like spread of small cell lung cancer to the brain that are causing symptoms (Untreated or symptomatic CNS metastases). 3. Prior treatment targeting T-cell co-stimulation, checkpoint pathways, and/or fuc-GM1. 4. Uncontrolled health conditions (certain lung diseases, heart diseases, infections, autoimmune diseases, other cancers, or a type of nerve damage called peripheral sensory neuropathy). 5. Other protocol-defined Exclusion criteria apply.

---

## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** BMS-986489 (fixed-dose combination) + Carboplatin + Etoposide administered every 3 weeks (Q3W) during the induction phase, followed by maintenance therapy with BMS-986489 administered every 4 weeks (Q4W). **Investigational Product Details:** Drug Name: etoposide | ATC Code: L01CB01 | Trade Names: Etopophos, Vepesid, Toposar. **Route of Administration:** Intravenous (IV) infusion **Duration of Treatment:** Until disease progression, unacceptable toxicity, death, or withdrawal of consent (maximum follow-up up to 5 years).

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care medications (e.g., anti-emetics, routine comorbidity treatments) not interfering with trial drug mechanisms. **Prohibited:** Other investigational agents, concurrent anti-cancer therapies, live vaccines, and immunosuppressive doses of systemic corticosteroids.

---

## 6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures |
| :--- | :--- | :--- |
| **Screening** | Day -28 to 0 | Informed Consent, Medical History, Baseline Imaging (CT/MRI), Labs, ECOG PS |
| **Baseline** | Day 1 | Randomization, First Dose (Induction Cycle 1) |
| **Interim** | Q3W (Induction) & Q4W (Maintenance) | Safety Labs, Efficacy Assessment, Tumor Scans, LCSS Questionnaires |
| **End of Study** | Up to 5 Years | Final Vital Signs, Exit Interview, IP Accountability, Overall Survival Follow-up |

---

## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Overall Survival (OS) - defined as the time from randomization to death due to any cause. **Measurement Method:** Clinical survival tracking evaluated over a time frame of up to 5 years.

**7.2 Secondary & Exploratory Endpoints** 1. Time to definitive deterioration (TTDD) based on the LCSS ASBI defined as the time from randomization until a definitive clinically meaningful decline ($\ge$ 10 point increase from baseline in LCSS ASBI score). 2. Number of participants with Adverse Events (AEs). 3. Number of Participants with Serious Adverse Events (SAEs). 4. Progression-Free Survival (PFS). 5. Objective Response Rate (ORR). 6. Duration of Response (DoR).

---

## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Continuous monitoring and grading per CTCAE during clinic visits, tracking immune-related adverse events (irAEs). **Serious Adverse Events (SAEs):** Must be reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) established to review safety and efficacy data periodically.

---

## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy (OS) and secondary efficacy outcomes. Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target sample size of $N=530$ is appropriately powered to detect a statistically significant shift/improvement in Overall Survival (OS) for the experimental arm vs. the active comparator (atezolizumab). **Handling of Missing Data:** Standard survival analysis right-censoring applies for patients lost to follow-up or still alive at the time of data cutoff.

---

## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of trial sites to ensure protocol adherence, accurate patient safety tracking, and data integrity. **Audit Trail:** Robust Electronic Data Capture (EDC) system with full audit trails, routine eCRF locking, and rigorous data clarification formats.

---

## 11. Study Identifiers & Governance

**Primary ID:** {{CA245-0001}} **Posting ID:** {{230841244700065827}} **Registry Number:** {{NCT06646276}} **Sponsor/Lead Organization:** {{Bristol Myers Squibb}} **Study Title:** {{TIGOS}} **Official Regulatory Title:** {{A Randomized, Double Blind, Multicenter Phase 3 Trial of BMS-986489 (BMS-986012+Nivolumab Fixed Dose Combination) in Combination With Carboplatin Plus Etoposide vs Atezolizumab in Combination With Carboplatin Plus Etoposide as First-Line Therapy in Participants With Extensive-Stage Small Cell Lung Cancer (TIGOS)}}

---

## 12. Clinical Framework (Study Specific)

**Disease Areas:** {{Extensive-Stage Small Cell Lung Cancer}} **Condition Name:** {{Extensive Stage Small Cell Lung Cancer}} **Preferred Name:** {{Small cell lung cancer extensive stage}} **Preferred UMLS Name:** {{Extensive-Stage Small Cell Lung Cancer}} **Semantic Type:** {{Neoplastic Process}} **Disease Definition:** {{Small cell lung carcinoma which has spread beyond one hemi-thorax and the regional lymph nodes.}} **MeSH Code:** {{D009369}} **SNOMED ID:** {{708064003}} **SNOMED Hierarchy:** {{188554009|Extensive stage small cell lung carcinoma|Extensive stage small cell lung carcinoma (disorder)}} **Diagnosis Threshold:** {{Histologically or cytologically documented ES-SCLC with $\ge$1 measurable lesion}} **Medical Methodology:** {{First-Line Platinum-based Doublet Chemotherapy + Immunotherapy}} **Optimization Target:** {{Overall Survival (OS) improvement via innate immune induction}}

---

## 13. Study Procedures & Methodology

**Management Pathway:** {{Standard oncology clinical pathway for ES-SCLC}} **Education Protocol (HCP):** {{Study protocol training & irAE management}} **Education Protocol (Patient):** {{Informed consent & IV treatment schedule guidance}} **Quality Audit Mechanism:** {{Clinical data monitoring & Independent Central Review (ICR)}}

---

## 14. Observation & Follow-Up Schedule

**Total Duration:** {{Up to 5 years (approx. 260 weeks)}} **Onsite Visit Cadence:** {{Every 3 weeks (Q3W) induction, Every 4 weeks (Q4W) maintenance}} **Remote Monitoring Frequency:** {{Continuous survival follow-up}} **Checklist Review Interval:** {{Tumor imaging assessment every 8-12 weeks}}

---

## 15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY] | | Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |